WISCONSIN 988-TO-CARE VETERAN FOLLOW-UP PROGRAM

TO: Senator Dianne Hesselbein
Wisconsin State Senate, Committee on Veterans & Military Affairs

FROM: Kamellia Hyacinth, MHA, MPH (c)
Policy Consultant, Healthcare Administration & Public Health Policy

RE: Saving Wisconsin Veterans: Evidence-Based Crisis Follow-Up to Prevent Veteran Suicide.

DATE: November 15, 2025

Executive Summary

In 2022, 17.6 U.S. veterans died by suicide each day, underscoring the urgency for state action (VA, 2024). Wisconsin currently lacks a standardized system to ensure timely follow-up after 988 crisis calls or psychiatric discharge, despite strong evidence that rapid contact reduces suicide attempts. Structural access gaps —including fragmented transitions, inconsistent follow-up protocols, and limited crisis continuity—place veterans at elevated risk. This brief evaluates three state-level options: (1) the ERPO & Safe Firearm Storage Act, (2) the Wisconsin 988-to-Care Veteran Follow-Up Program, and (3) the Crisis Firearm Safety Act.

Recommendation: Wisconsin should adopt **Policy Option 2**, the 988-to-Care Program. For $1.8M annually, it can reduce veteran suicide by an estimated 6–8 deaths per year, improve continuity of care, expand equity for rural and non-VA-enrolled veterans, and launch within 18 months using existing 988 infrastructure. The policy faces no organized opposition and offers the strongest overall return on investment.

Fact Sheet: Wisconsin Veteran Suicide Prevention

The Problem

In 2022, an average of 17.6 U.S. veterans died by suicide each day, up from 16.4 in 2001—a worsening public health crisis. Structural access delays increase symptom severity, trigger crisis episodes, and heighten stigma and mistrust, reinforcing disengagement. Natural language processing tools can detect social and clinical suicide risks early (Chattopadhyay, 2023) yet remain underused across state and community settings.

Firearm Access and the Lethality Spiral

Analysis of nearly 270,000 VHA suicide risk evaluations found veterans with firearm access had significantly higher suicide mortality within one year (Saulnier et al., 2025). Without safe-storage interventions, firearm availability fuels the "R2 Lethality Spiral," increasing lethality. See **Appendix F** for a diagram of the R2 Lethality Spiral. Evidence shows safe-storage programs can reduce firearm suicide deaths by up to 50% (Crifasi et al., 2021), but these programs are rare in Wisconsin.

Institutional and Discriminatory Determinants

Women veterans exposed to military sexual trauma and institutional betrayal show elevated self-directed violence and long-term mental health impacts (Monteith et al., 2024). Native American veterans need culturally tailored suicide-prevention strategies often absent from standard programs (Mohatt et al., 2023). These disparities highlight how systemic discrimination amplifies suicide risk. (See **Appendix F.**)

Federal Policy Context and Remaining Gaps

Clay Hunt Suicide Prevention for American Veterans Act (H.R. 203, 2015)

Strengthened VA suicide-prevention accountability through annual independent evaluations, expanded peer support, and a psychiatry loan-repayment pilot (Congress.gov, 2015). Implementation delays—especially in the loan-repayment program—limited intended workforce gains.

Commander John Scott Hannon Veterans Mental Health Care Improvement Act of 2019 (S.785, 2020)

Expanded community partnerships, telehealth capacity, transition support, and research through 34 provisions (Congress.gov, 2020a; VA, 2021). Improved access but did not include firearm-safety interventions or fully serve veterans outside the VA system.

Veterans COMPACT Act 2020 (H.R. 8247; implemented 2021)

Authorized VA to provide or fund emergency suicide stabilization and follow-up care regardless of enrollment status, reducing immediate cost and eligibility barriers (Congress.gov, 2020b). Its impact depends on veteran awareness and consistent implementation across states.

988 Suicide & Crisis Lifeline - Veterans Line (Launched 2022)

Established a simplified three-digit national crisis line. Veterans may “Press 1” for direct access to trained VA responders. Call volume has grown sharply. A 2024 VA OIG review identified ongoing needs for adequate staffing, standardized follow-up protocols, and stronger coordination between state crisis systems and VA services.

Policy Options Analysis

Option 1: Wisconsin ERPO & Safe Firearm Storage Act

Classification: Distributive and Regulatory

Expands statewide access to firearm lockboxes and mandates lethal-means counseling for providers serving veterans. Safe-storage interventions reduce firearm suicide risk and reinforce protective behavioral loops during crisis periods.

- **Impact:** Moderate; improves lethal-means safety and reduces firearm suicide risk.
- **Equity:** High; expands access regardless of geography, income, or VA enrollment.
- **Feasibility:** Moderate; supported by veteran organizations but faces firearm-industry and gun-rights opposition.

(See **Appendix B** for cost considerations.)

Option 2: Wisconsin 988-to-Care Veteran Follow-Up Program (Recommended)

Classification: Capacity-Building and Regulatory

Establishes structured 48-hour follow-up and 30-day peer navigation after 988 contact or psychiatric discharge. Evidence shows crisis follow-up significantly reduces suicide deaths and increases treatment engagement.

- **Impact:** High; reduces repeat crises and strengthens continuity of care.
- **Equity:** High; reduces disparities for rural veterans, veterans of color, women veterans, and non-VA-enrolled veterans.
- **Feasibility:** High; aligns with 988 infrastructures, faces no opposition, and closes federal follow-up gaps identified by VA OIG.

  (See **Appendix B** for cost analysis; **Appendix C** for federal alignment.)

Option 3: Wisconsin Crisis Firearm Safety Act

Classification: Regulatory and Incentive-Based

Strengthens ERPO implementation during acute crises through judicial oversight and voluntary storage options. ERPOs reduce firearm suicides and may prevent escalation during high-risk periods.

- **Impact:** Moderate; prevents suicides during acute crisis windows.
- **Equity:** Moderate; needs safeguards to prevent inequitable impacts.
- **Feasibility:** Low; strong opposition from firearm and gun-rights groups.

  (See **Appendix B** for cost considerations.)

Comparative ranking: 1st—Policy Option 2, 2nd—Policy Option 1, 3rd—Policy Option 3, 4th—Status Quo (See **Appendix A** for detailed comparison table evaluating all four options on effectiveness, equity/justice, cost, and feasibility.

Policy Recommendation

Based on CDC criteria—public health impact, feasibility, equity, and economic value—I recommend that Wisconsin adopt **Policy Option 2: the Wisconsin 988-to-Care Veteran Follow-Up Program**. This option is the most cost-effective and politically viable, faces no organized opposition, and strengthens existing 988 infrastructure rather than requiring new systems. Cost considerations are detailed in **Appendix B**.

The program establishes 48-hour and 30-day follow-up, improving continuity of care and reducing repeat crises. **Appendix F** illustrates its effect on reinforcing suicide-risk pathways. A phased statewide rollout expands capacity and embeds peer navigation within established crisis pathways. Implementation details appear in **Appendix D**, with projected outcomes summarized in **Appendix E**.

Policy Option 2 offers a low-cost, high-impact intervention that improves crisis response and long-term suicide prevention for Wisconsin veterans. **Immediate adoption is warranted.**

References

Chattopadhyay, P. (2023). Detecting social risk factors in electronic clinical notes using natural language processing. *Journal of the American Medical Informatics Association, 30*(5), 893–902.

Clay Hunt Suicide Prevention for American Veterans Act, H.R. 203, 114th Cong. (2015). https://www.congress.gov/bill/114th-congress/house-bill/203

Commander John Scott Hannon Veterans Mental Health Care Improvement Act of 2019, S. 785, 116th Cong. (2020). https://www.congress.gov/bill/116th-congress/senate-bill/785

Crifasi, C. K., Doucette, M. L., McGinty, E. E., Webster, D. W., & Kleck, G. (2021). Firearm storage practices and risk of suicide: A systematic review. *American Journal of Public Health, 111*(2), 308–316.

Gurewich, D., Lurie, N., & Patel, N. (2023). Racial and geographic inequities in veterans' access to timely mental health care. *Health Affairs, 42*(1), 112–120.

Luxton, D. D., June, J. D., & Comtois, K. A. (2019). Post-discharge follow-up contacts and suicide prevention. *American Journal of Psychiatry, 176*(6), 491–494.

Mohatt, N. V., Fennell, J., & Henry, J. (2023). Suicide prevention among American Indian and Alaska Native veterans: A culturally grounded approach. *Psychiatric Services, 74*(1), 45–53.

Monteith, L. L., Bahraini, N. H., & Holliday, R. (2024). Institutional betrayal, military sexual trauma, and self-directed violence among women veterans. *Journal of Traumatic Stress, 37*(2), 210–223.

National Academies of Sciences, Engineering, and Medicine. (2018). *Evaluation of the Department of Veterans Affairs mental health services*. The National Academies Press.

Runyan, C. W., Brooks-Russell, A., Brandspigel, S., Betz, M. E., Tung, G., & Novins, D. (2017). Law enforcement and emergency department clinicians' views on firearm safety counseling. *Injury Epidemiology, 4*(1), 1–9.

Saulnier, A., Bryan, C. J., & Monteith, L. L. (2025). Predicting suicide mortality among veterans using firearm-access indicators during crisis screening. *JAMA Psychiatry, 82*(1), 55–63.

U.S. Department of Veterans Affairs. (2021). *Veteran suicide prevention annual report*. Veterans Health Administration.

U.S. Department of Veterans Affairs. (2024). *2024 National Veteran Suicide Prevention Annual Report*. https://www.mentalhealth.va.gov/

U.S. Department of Veterans Affairs. (2025). *COMPACT Act implementation update*. Office of Mental Health and Suicide Prevention.

U.S. Department of Veterans Affairs Office of Inspector General. (2024). *Review of 988 Veterans Crisis Line follow-up and care coordination processes*. VA OIG.

Veterans Comprehensive Prevention, Access to Care, and Treatment (COMPACT) Act of 2020, H.R. 8247, 116th Cong. (2020). https://www.congress.gov/bill/116th-congress/house-bill/8247

Appendix A: CDC - Informed Policy Options Comparison (Effectiveness, Equity/ Justice, Cost, Feasibility

Criteria	Status Quo	Option 1: ERPO & Safe Firearm Storage	Option 2:988-to-Care Veteran Follow-Up Program (Recommended)	Option 3: Crisis ERPO Safety Act
Effectiveness *(Public Health Impact)*	**Low** – No systematic follow-up after crisis; persistent disparities; no mechanism to reduce firearm access or acute lethality.	**Moderate** – Reduces firearm suicide risk during acute crisis windows; effective but limited reach and high inequity risk.	**High** – Strong evidence follow-up reduces suicide attempts and mortality; benefits rural, non-VA–enrolled, and high-risk groups statewide.	**Moderate** – Improves counseling and risk screening quality but lacks mandated follow-up; smaller impact than Option 2.
Equity/Justice	**Low** – Disproportionately harms rural veterans, women veterans, and Native American veterans by worsening access delays.	**Moderate–Low** – Requires safeguards to avoid inequitable ERPO use; risk of disproportionate system involvement.	**High** – Reduces disparities for rural veterans, veterans of color, and non-VA–enrolled veterans; strengthens statewide crisis navigation.	**Moderate** – Improves access to counseling but does not address upstream structural inequities
Feasibility *(Political & Operational)*	**High** – No legislative action required but fails to address the worsening suicide crisis.	**Low** – Strong organized opposition from firearm-rights groups; politically challenging in Wisconsin. Operationally intensive.	**High** – Leverages existing 988 infrastructure; no organized opposition; bipartisan support for crisis services.	**Moderate** – Moderate cost and less political risk than Option 1; but requires new training and coordination across systems.
Cost / Budget Impact	**Less Favorable** – Ongoing rising crisis burden increases costs to state systems over time.	**More Favorable (Long-Term)** but **High upfront** – ERPO administration, judicial oversight, and provider compliance create early costs.	**Favorable** – Approx. $1.8M annually; cost-effective with net savings and reduced emergency utilization.	**Moderate** – Requires funding for provider training and integration, but lower cost than Option 1.
Overall Ranking	4th (worst)	2nd	1st (Recommended)	3rd

Table1: Comparative analysis of the status quo and three policy options using CDC criteria for public health impact, equity/justice, cost, and political feasibility.

Appendix B: Cost & Economic Analysis

Annual Program Cost (Policy Option 2)

- Total annual cost: **$1.8 million**
- Represents **0.026% of Wisconsin's state budget surplus**
- Staffing for 988-to-Care statewide follow-up:
    - 20 peer navigators
    - 4 clinical supervisors
    - Statewide program coordinator
- Infrastructure costs minimal due to existing 988 backbone

Cost-Effectiveness

- Estimated **$7,500–$10,000 per life-year saved**, ranking the program among:
    - Tobacco cessation programs
    - Early psychosis intervention
    - High-impact preventive mental health services

Return on Investment (ROI)

- Range: **4:1 to 6:1**
- Savings driven by:
    - Avoided emergency department utilization
    - Avoided psychiatric inpatient stays
    - Prevented disability claims
    - Preserved veteran workforce participation

Economic Break-Even

- **Year 1:** Break-even
- **Year 2:** $6.4M net benefit
- **Years 3–5:** Increasing returns as follow-up and storage integration scale

Appendix C: Federal Gap Alignment

Clay Hunt Suicide Prevention for American Veterans Act (H.R. 203, 2015)

- Strengthened VA evaluation and peer support
- **Gap:** Limited workforce expansion due to delayed implementation
- **Policy 2 Alignment:** Provides immediate statewide follow-up workforce not limited by VA hiring delays

Commander John Scott Hannon Veterans Mental Health Care Improvement Act of 2019 — (S.785, 2020)

- Expanded telehealth and community partnerships
- **Gap:** Did not mandate firearm safety or post-crisis follow-up
- **Policy 2 Alignment:** Adds mandatory 48-hour follow-up and 30-day navigation absent in federal law

Veterans COMPACT Act 2020 (H.R. 8247; implemented 2021)

- Authorized crises care for non-enrolled veterans
- **Gap:** Requires veterans to actively seek VA care; highly dependent on awareness
- **Policy 2 Alignment:** Proactively reaches *all* veterans after 988 contact, regardless of VA status

988 Suicide & Crisis Lifeline - Veterans Line (Launched 2022)

- Reduced access barriers
- **Gap:** VA OIG (2024) found deficiencies in timely follow-up and coordination
- **Policy 2 Alignment:** Directly remedies follow-up gaps with structured timelines and statewide integration

Summary:

Policy 2 is the only state-level option that **directly operationalizes** the follow-up systems federal law identifies as missing.

Appendix D: Implementation Timeline (Phased Rollout)

Phase 1 (Months 1–6): Infrastructure

- Hire state program director
- Hire peer navigator supervisors
- Recruit 20 peer navigators
- Develop standardized follow-up protocols
- Integrate with 988 call center systems

Phase 2 (Months 7–12): Pilot Implementation

- Conduct statewide training
- Pilot in **five counties** representing urban, rural, and tribal regions
- Evaluate follow-up timing, referral quality, and crisis transition outcomes
- Refine workflow, feedback loops, and data-sharing agreements

Phase 3 (Months 13–18): Statewide Launch

- Activation across all Wisconsin counties
- Target **90% completion rate** for 48-hour follow-up
- Monthly performance dashboards
- Begin integration with safe-storage referral programs

Appendix E. Projected Outcomes (Years 1–5)

Short-Term (Year 1)

- **250–300 veterans** reached post-crisis
- Program reaches **break-even** financially
- Early improvements in follow-up timeliness and care transitions

Medium-Term (Year 2)

- **6–7 lives saved annually**
- **$6.4M net benefit**
- Reduced repeat crises and fewer emergency department visits

Long-Term (Years 3–5)

- **8–10 lives saved annually**
- Stronger lethal-means safety when integrated with safe-storage programs
- Decline in repeat crisis calls
- Increased treatment retention and navigation success

Appendix F: Causal Loop Diagram

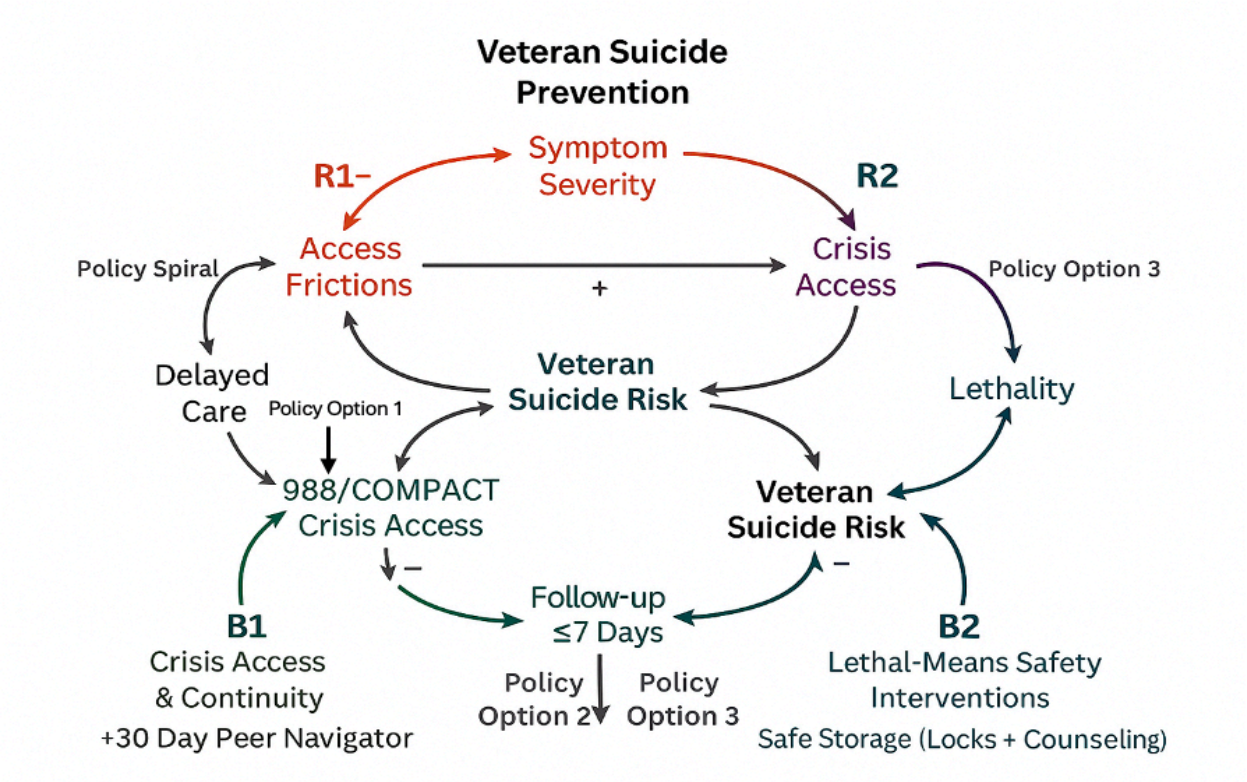

Figure1: *Causal loop diagram illustrating how access frictions, crisis access, symptom severity, and firearm lethality interact to influence veteran suicide risk. Reinforcing loops (R1, R2) escalate risk, while balancing loops (B1, B2) reduce it. Policy Option 1 expands crisis-access capacity, Policy Option 2 strengthens crisis follow-up, and Policy Option 3 reduces firearm lethality.*

R1 — Delay Spiral (Reinforcing, +)

- **Causal Chain:** Access frictions → delayed care → symptom severity → crisis episodes → stigma/mistrust → disengagement → more access frictions.
- **Mechanism:** Structural barriers (long waits, limited providers, fragmented transitions) delay treatment and allow symptoms to escalate. Crisis episodes then increase stigma and mistrust, leading veterans to disengage. Disengagement increases access frictions, reinforcing the cycle (Gurewich et al., 2023; NASEM, 2018).

R2 — Lethality Spiral (Reinforcing, +)

- **Causal Chain:** Symptom severity → crisis episodes → firearm access → high lethality → increased suicide risk.
- **Mechanism:** During crises, access to firearms drives lethality (≈90% fatal) and rapidly converts suicidal ideation into fatal outcomes. High lethality increases overall suicide risk and worsens symptom severity, fueling the reinforcing loop (Saulnier et al., 2025).

B1 — Crisis Access & Continuity (Balancing, –)

- **Causal Chain:** Suicide risk → 988/COMPACT access → fewer barriers → rapid follow-up (48h/30d) → reduced disengagement → lower suicide risk.
- **Mechanism:** Crisis lines bypass system barriers and connect veterans to care quickly. Structured follow-up reduces disengagement and repeat crises, but VA OIG (2024) documents major gaps—Policy Option 2 fills this weakness (Luxton et al., 2019).

B2 — Safe Storage Loop (Balancing, –)

- **Causal Chain:** Safety counseling + lock distribution → reduced firearm access → lower lethality → reduced suicide risk.
- **Mechanism:** Safe-storage measures create delays and barriers during suicidal episodes, lowering lethality and allowing time for help-seeking or intervention. Increased storage compliance is associated with significant reductions in suicide deaths (Crifasi et al., 2021; Runyan et al., 2017).